



Designing a Resource Kiosk for Ovarian Cancer Survivors

A healthcare UX and service design project exploring how survivors can access trusted support, education, and community after treatment.

Transitioning out of cancer treatment can bring a new kind of uncertainty. During treatment, patients often have structured care plans and regular guidance from providers. After treatment ends, survivors may need to navigate a more self-directed journey - finding reliable information, emotional support, and community resources on their own.

Through Design for America, my team explored how we might better support ovarian cancer survivors as they transitioned from treatment into survivorship. Our final concept was a private,

accessible resource kiosk designed for an oncology care setting, where survivors could discover personalized resources, support groups, and community connections after their final appointment.

Project Snapshot

Project / Client: Design for America, ovarian cancer survivorship support

Timeline: September 2018 – May 2019

Role: UX Researcher, UX/UI Designer

Team / Stakeholders: 5 student designers and researchers, gynecologic oncologists, patients / survivors

Problem Space: Supporting ovarian cancer survivors as they transitioned from active treatment into survivorship and needed trusted resources, support, and connection

Methods: Secondary research, provider interviews, patient / survivor interviews, synthesis, ideation, wireframing, low-fidelity prototyping, physical prototyping

Tools: Figma, physical prototyping materials, Miro, Google Workspace

Outputs: Research findings, design requirements, kiosk interface prototype, physical kiosk model, final Figma prototype, personalized resource printout, and [final presentation](#)

Outcome / Impact: Designed a private, accessible resource kiosk concept for an oncology care setting, focused on support, connection, education, privacy, and professional resource validity

Focus Areas: Healthcare UX, patient experience, service design, accessibility, privacy, sensitive research

About Design for America

Design for America is a student-led design organization focused on applying human-centered design to social challenges on college campuses and in local communities.

For this project, our team focused on women's healthcare, specifically the transition from ovarian cancer treatment into survivorship.

The Challenge

Our team began with a broad interest in women's healthcare and the gaps people experience when navigating complex care systems. We chose to focus on ovarian cancer survivorship because one of our team members had a personal connection to a community of ovarian cancer survivors, which helped us approach the topic with both care and curiosity.

Through early research, we learned that survivors often face a difficult transition after treatment. During active treatment, care is structured and provider-led. But once treatment ends, survivors may be left to find resources, support networks, and education more independently.

This led us to our core design question:

How might we better support ovarian cancer survivors in their transition from treatment into survivorship?

My Role

As a UX researcher and UX/UI designer, I contributed to the research, synthesis, ideation, and prototyping phases of the project.

I helped the team understand the needs of survivors and providers, translate research findings into design requirements, and develop a kiosk concept that prioritized privacy, accessibility, and trustworthy information. I also supported the interface design and physical prototype development as the concept evolved from early wireframes into a final presentation.

Research Approach

We began with secondary research to understand ovarian cancer treatment, survivorship, and the broader healthcare context. From there, we conducted interviews with gynecologic oncologists, patients, and survivors to understand what people needed during the post-treatment transition.

Our research focused on questions like:

What changes after treatment ends?

Where do survivors go for information and support?

What resources feel trustworthy?

How do patients and providers think about survivorship planning?

What emotional, social, and educational needs remain after treatment?

Because this was a sensitive healthcare context, we approached the research carefully and focused on understanding survivors' needs without assuming that one product could solve the full complexity of their experience.

Key Research Findings

1. Survivorship created a transition gap.

We learned that the post-treatment phase could feel less structured than active treatment. During treatment, patients often have a clear plan guided by their gynecologic oncologist. After treatment, survivors may need to take on more responsibility for planning their own next steps.

This created an opportunity to design support for the moment when patients move from provider-led care into more self-directed survivorship.

2. Patients and providers were concerned about information validity.

One provider shared that they spent a lot of time telling patients where not to search. That quote stuck with our team because it revealed a major tension: survivors wanted information, but not all available information felt accurate, relevant, or trustworthy.

This insight pushed us to prioritize professionally validated resources rather than simply creating another place to browse generic health information.

3. Survivors wanted education after treatment.

Survivors expressed a desire to better understand life after treatment, including what resources were available, what to expect, and how to navigate the next stage of care.

This helped us frame education as an ongoing survivorship need, not something that ends when treatment ends.

4. Support and connection mattered deeply.

Many survivors valued support groups, especially ones that reflected their interests and lifestyles. For example, one interviewee referenced a cycling group as a meaningful source of positivity and connection.

This helped us understand that support was not only clinical or informational. It was also social, personal, and identity-based.

5. Treatment can affect interpersonal relationships.

Survivorship is not experienced in isolation. Treatment can change relationships with partners, family, friends, and communities. This reinforced the need to design for emotional and social support, not just information access.

Defining the Design Requirements

After reviewing our interviews and synthesizing the research, we narrowed our design requirements around three core needs:

Support

Survivors valued support groups and communities, especially when those groups reflected their personal interests and lived experiences.

Connection

Survivors wanted ways to connect with others who understood what they were going through, particularly during the transition out of active treatment.

Education

Survivors needed reliable, relevant, and accessible resources that had been validated by professionals in the field.

These three needs became the foundation for our concept.

Ideation

We explored several possible ways to support ovarian cancer survivors during the post-treatment transition. Because our research pointed to needs around support, connection, and education, we intentionally brainstormed beyond a single screen-based solution.

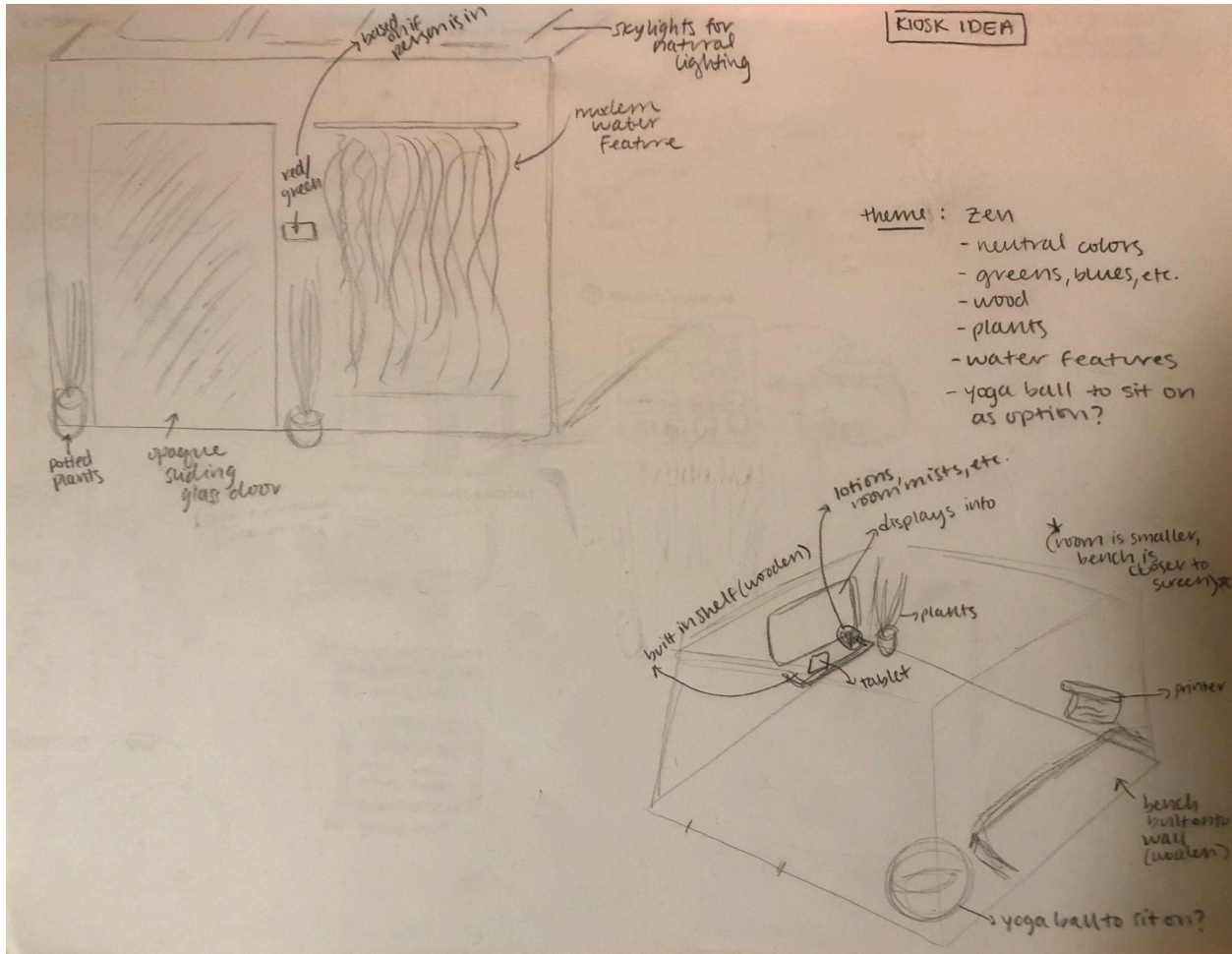
Early ideas included printed resource takeaways, journals, invitations, calendars, mobile information displays, interactive prompts, and different types of physical spaces. These sketches helped us think through not only what information survivors might need, but where and how they might feel comfortable receiving it.

As we evaluated ideas, we kept returning to one important question:

Where would survivors feel comfortable accessing sensitive resources?

That question helped us narrow from a wide set of resource delivery concepts toward a more private, place-based solution. We eventually decided to design a digital resource kiosk for an oncology care setting. The kiosk would be used after a patient's final appointment and would help them find personalized resources, support groups, and communities based on their needs and interests.

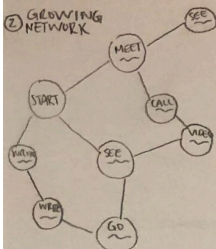
We chose a kiosk because the oncology setting was already a trusted and familiar environment for many users. Instead of asking survivors to search for support on their own later, the kiosk would meet them at a meaningful transition point in their care journey.



1 Brochure

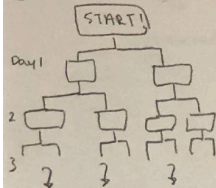


2 GROWING NETWORK

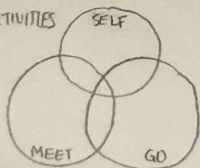


incentives?
- gift basket

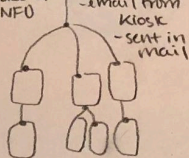
3 TREE



4 ACTIVITIES

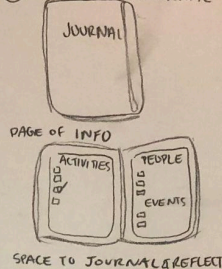


5 MOBILE OF INFO

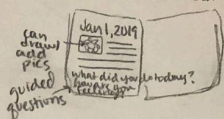


- email from kiosk
- sent in mail

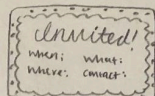
6 CUSTOMIZED JOURNAL



SPACE TO JOURNAL & REFLECT

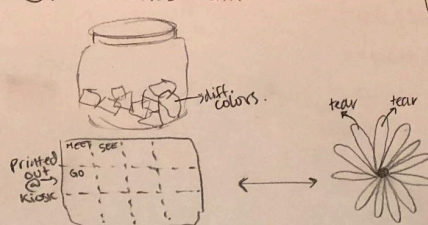


7 INVITATIONS (sent every other week for an event?)



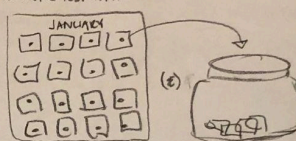
Fb group, too?

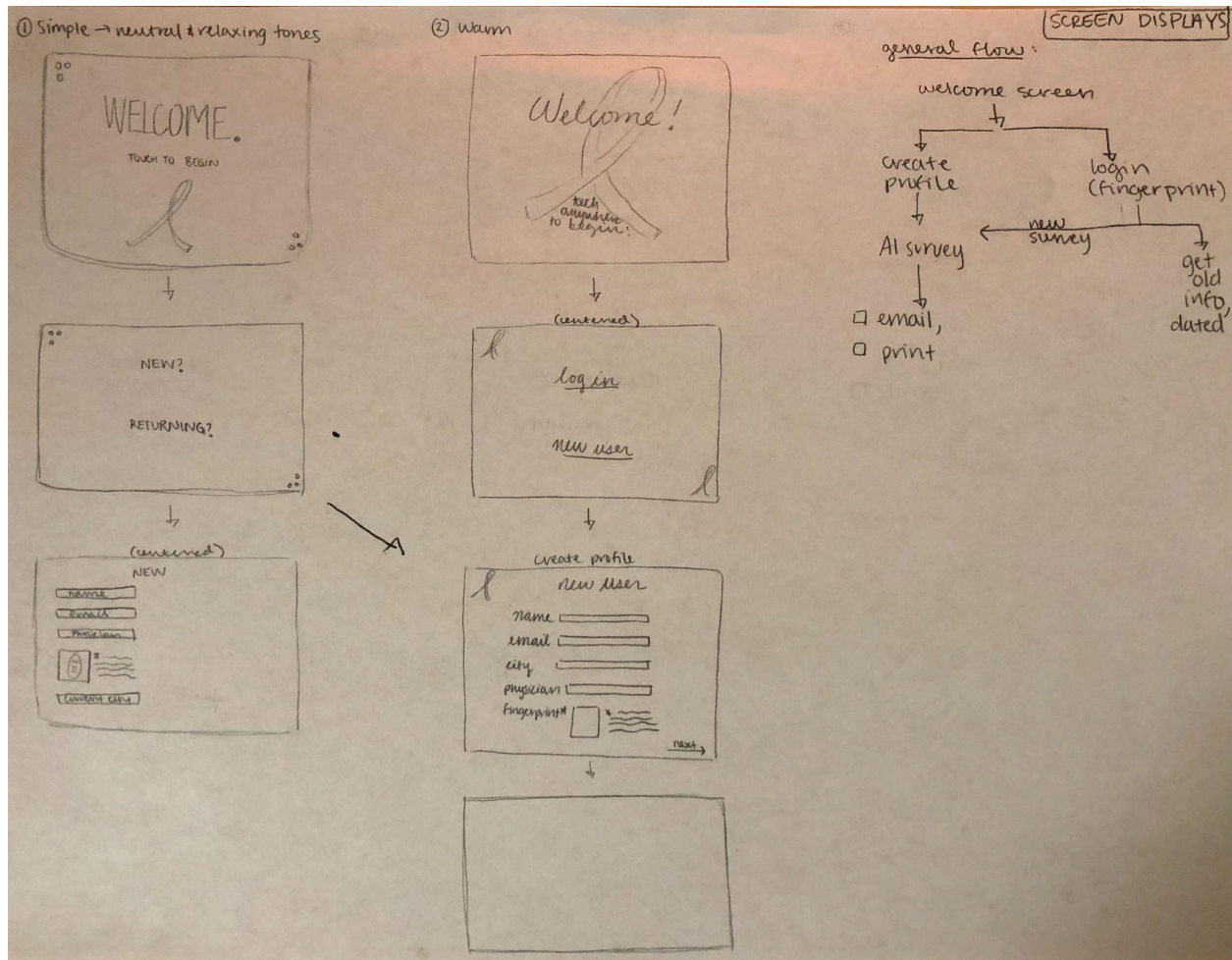
8 TEAR OFF & ADD TO JAR



INFORMATION DISPLAYS

9 ADVENT CALENDAR





Caption: Before converging on the private resource kiosk, we explored a wide range of ways survivors could access support, education, and community resources after treatment. These sketches helped us think through the full experience: physical environment, emotional comfort, privacy, accessibility, information delivery, and what survivors could take with them after leaving the care setting.

Concept: A Private Resource Kiosk

The final concept was a digital resource kiosk located in the waiting room or office area of an oncology care setting.

The kiosk would allow survivors to:

- Answer a few questions about their needs and interests
- Receive personalized survivorship resources
- Discover support groups and community options

Access professionally validated education
Print a personalized resource sheet to take home

The goal was not to replace providers or support groups. Instead, the kiosk acted as a bridge between clinical care and life after treatment.

Low-Fidelity Prototyping

Our early prototype included both a digital interface and a physical clay model of the kiosk.

For the interface, we explored how users might move through a short resource-finding experience. We wanted the flow to feel simple, respectful, and personalized, especially for users who may already feel overwhelmed at the end of treatment.

For the physical kiosk, we initially explored a larger sculptural form that could create privacy and visually signal the purpose of the kiosk. However, we later realized the first design was too large for a gynecologist's office or oncology waiting room. We also wanted the screen to be less visible to others so users could feel more comfortable, safe, and respected while engaging with sensitive information.

Connections

Connections

Connections

NAME

Connecto

GOALS

Resources

Support

CONNECTIONS



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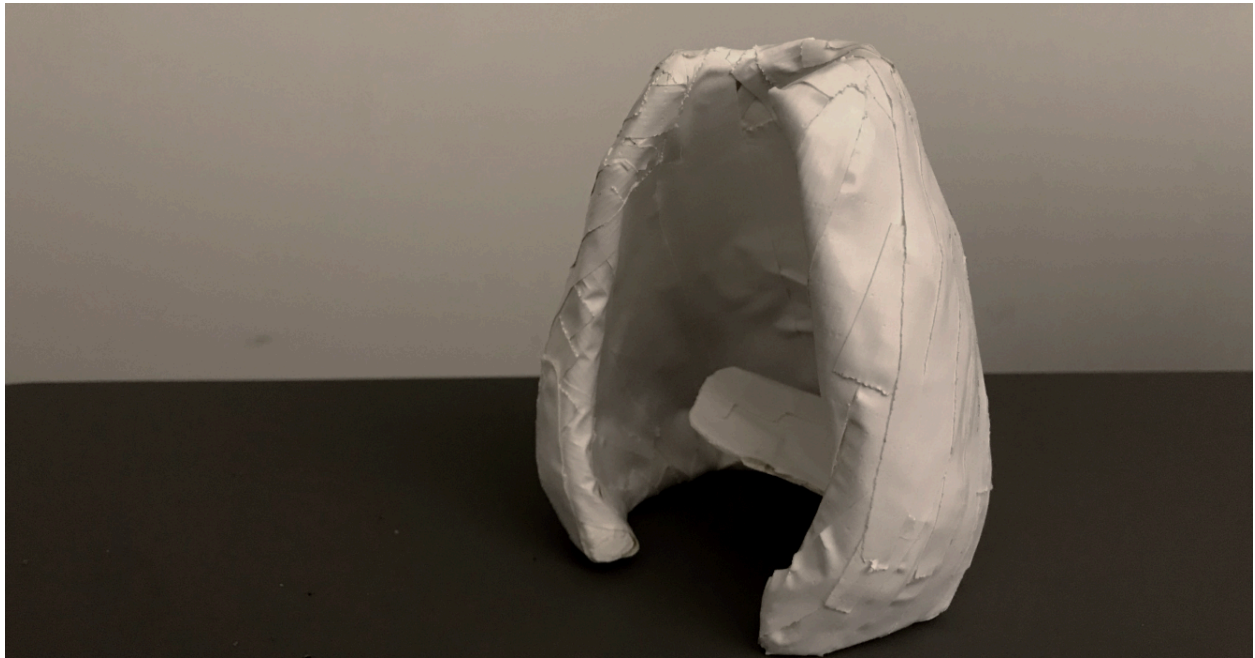
caroline.sands@gmail.com



ADD A NEW CONNECTION



Caption: *Our first prototype explored how the kiosk could provide personalized resources while creating a private, supportive interaction.*



Caption: *We moved away from the initial kiosk shape after identifying concerns around size, screen visibility, and fit within a medical office.*

Iterating for Privacy, Accessibility, and Feasibility

As we refined the kiosk, we focused on three priorities:

Privacy

Because users would be engaging with personal health-related information, the kiosk needed to feel discreet and protected. We adjusted the form so the screen would be less visible to others in the room.

Accessibility

The kiosk needed to accommodate different bodies, mobility needs, and comfort levels. In the final prototype, the curved walls created a more enclosed space, and a sliding panel allowed for more leg or wheelchair space.

Feasibility

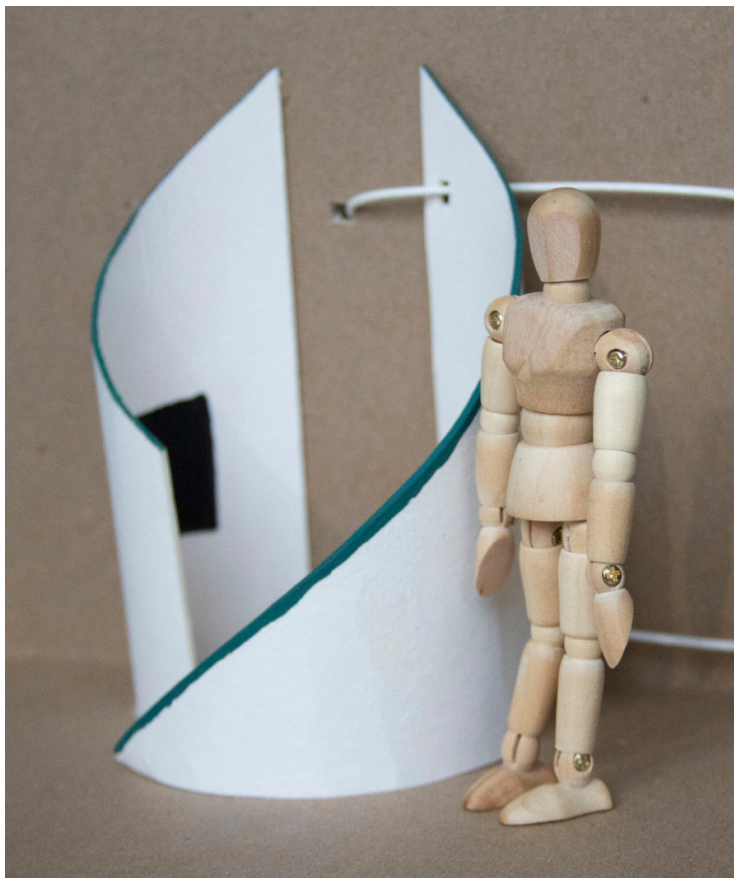
The kiosk also needed to be realistic for a medical office environment. We reduced its overall size and designed it to use the existing wall, which helped reduce material needs and make the concept more feasible for a gynecology or oncology office.

Final Prototype

Our final prototype included a refined physical kiosk model, a Figma interface, and a personalized resource printout.

The kiosk was designed to create a safe, semi-private space for survivors to explore resources at their own pace. The interface guided users through a short set of prompts and generated personalized recommendations based on their needs and interests.

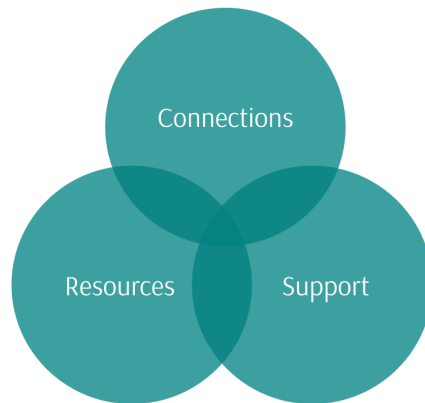
At the end of the experience, users could print a resource sheet to take with them. This was important because it allowed survivors to leave with something tangible that they could revisit later, share with a caregiver, or bring into a future conversation with a provider.



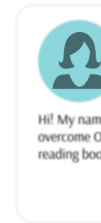
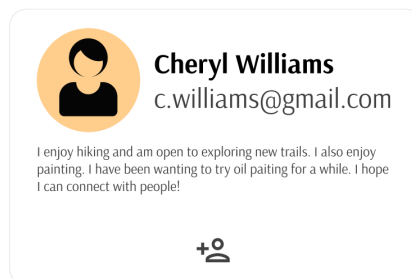
Caption: *The final kiosk design prioritized privacy, accessibility, and feasibility within an oncology care setting.*

Aurora

 Ann Wilson



 Connections



Caption: *The printed resource sheet gave survivors a tangible takeaway with personalized support, education, and community options.*

Why This Solution Fit the Problem

The kiosk concept worked because it responded to several research findings at once.

Survivors wanted reliable information, so the kiosk focused on professionally validated resources. They wanted support and connection, so it helped surface communities and support groups based on personal interests. They were navigating a transition out of structured care, so the kiosk was designed for the moment after a final appointment, when survivors may be thinking about what comes next.

The physical form of the kiosk also mattered. Because this was a sensitive healthcare experience, we could not treat the interface and the environment separately. Privacy, screen visibility, wheelchair space, and placement in a trusted care setting were all part of the user experience.

Presentation

At the end of the project, our team presented the final kiosk concept, interface prototype, physical model, and resource printout.

The final presentation showed how the kiosk could support survivors through a personalized, private, and accessible experience that connected them to education, support, and community resources after treatment.

Feel free to check out my team's final presentation [here!](#)

Reflection

This was one of my earliest UX projects, and it introduced me to the role user research can play in designing for sensitive, emotionally complex experiences.

The project taught me that meaningful design starts with understanding the user's context, not jumping straight to a solution. In this case, the challenge was not just that survivors needed more information. It was that they needed trustworthy, relevant, and accessible support during a transition that could feel emotionally and logistically uncertain.

I also learned how important it is to align on design principles before moving too far into execution. Once our team clarified that the experience needed to prioritize support, connection, and education, our design decisions became more focused and easier to evaluate.

Finally, this project helped me understand how physical context shapes digital experiences. The kiosk was not just a screen. Its placement, size, privacy, accessibility, and physical form all influenced whether users would feel safe and comfortable using it.

What I'd Do Differently Today

If I were approaching this project now, I would spend more time validating the concept with survivors, caregivers, and healthcare providers before finalizing the kiosk form.

Specifically, I would want to better understand:

Whether survivors would feel comfortable using a kiosk in a medical office
Whether the end-of-treatment appointment is the right moment to introduce survivorship

resources

How users would want resources personalized

What level of provider validation would make the information feel trustworthy

Whether a printed resource sheet, digital follow-up, or both would be most useful

I would also explore service design questions more deeply, such as who would maintain the resource database, how providers would introduce the kiosk, and how the experience could connect to existing survivorship care planning.

Even with those limitations, this project gave me an early foundation in research-led design, healthcare UX, accessibility, and designing for trust in sensitive contexts.

Short Version for the Project Card

Designing a Resource Kiosk for Ovarian Cancer Survivors

Designed a healthcare UX concept through Design for America to support ovarian cancer survivors transitioning from treatment into survivorship. Through provider and survivor research, our team identified needs around trusted education, support, and connection, then prototyped a private, accessible resource kiosk for an oncology care setting.

Tags

Healthcare UX · Service Design · UX Research · Sensitive Research · Patient Experience · Survivorship Support · Accessibility · Privacy · Resource Navigation · Prototyping · Figma · Physical-Digital Experience